NCE EXAM PREP

Practice Exam 08

Domain 2

Intake & Assessment

30 Practice Questions | 40 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 02. Use the suggested 40-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

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PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

> A client brings a close friend to intake for support. The friend begins answering every question and asks to receive appointment reminders and copies of future assessment results. The client has not yet stated what involvement is wanted.

What should the counselor do?

A) Accept the friend's requests because bringing a support person grants access to future information

B) Exclude the friend immediately without asking the client about preferences

C) Speak directly with the client about the friend's desired role, privacy boundaries, and any specific permission before involving the friend

Question 2

> A client has a history of a three-week major depressive episode. The client also reports a distinct five-day period of elevated mood, decreased need for sleep, increased talkativeness, and unusual goal-directed activity that was observable to others but caused no marked impairment, psychosis, or hospitalization. There has never been a manic episode, and substances and medical causes have been ruled out.

Which diagnosis is most consistent with this history?

A) Bipolar I disorder

B) Bipolar II disorder

C) Persistent depressive disorder

Question 3

> An adult with a mild intellectual disability arrives with a sister, who says, “I always sign things for him.” No documentation identifies the sister as a legal decision-maker. The client communicates preferences clearly but needs extra time and simplified wording to understand written material.

How should the counselor handle informed consent?

A) Have the sister sign immediately because an intellectual disability eliminates the client's ability to consent

B) Ask the sister to leave and use the standard form without any communication adjustments

C) Provide accessible explanations, assess the client's understanding and decision-making ability, and verify whether anyone has legal authority before involving a substitute decision-maker

Question 4

When asked why work has become difficult, a client provides excessive detail about several coworkers and office routines. The information remains logically connected, and the client eventually answers the question after repeated redirection.

Which mental status description best fits the client's thought process?

A) Flight of ideas

B) Circumstantial

C) Thought blocking

Question 5

A single parent who works rotating overnight shifts requests counseling at a community agency. The parent has missed appointments elsewhere because available times conflicted with work and child care. This agency offers some evening sessions and limited on-site child care.

What is the most appropriate intake response?

A) Refer the client elsewhere because prior missed appointments indicate poor readiness for counseling

B) Explore scheduling and child-care barriers with the client and determine whether the agency's available supports make services workable

C) Assign a standard daytime appointment so the counselor can evaluate commitment before discussing alternatives

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Question 6

A person contacts a counseling-only clinic requesting evaluation and management of psychiatric medication. The clinic has no prescriber but can provide counseling and maintains referral relationships with medical providers.

What is the most appropriate intake action?

A) Explain the clinic's scope, check for urgent concerns and interest in counseling, and offer an appropriate prescribing referral

B) Schedule the person with a counselor and imply that the counselor can adjust medication after the first session

C) Reject the request without assessing urgency or explaining available referral and counseling options

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Question 7

What does a score on a criterion-referenced assessment primarily indicate?

A) Performance in relation to defined knowledge or skill standards

B) Rank compared with a representative norm group

C) Consistency of scores across different raters

Question 8

A multilingual middle-school student is referred for possible learning difficulties after one year of instruction in English. The school proposes using only an English cognitive test that lacks appropriate norms for students with the learner's language history.

Which assessment approach is most appropriate?

A) Use multiple methods and qualified language support while considering educational opportunity, language development, culture, and measures appropriate for the student's background

B) Use the English test alone because cognitive scores are unaffected by language proficiency or educational experience

C) Delay any assessment indefinitely until the student speaks English without an accent

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Question 9

A college student seeks assessment for social anxiety. The student describes difficulty interpreting social cues since early childhood, intense distress when routines change, longstanding sensory sensitivities, and repeated highly focused interests. Anxiety increased only after peers began criticizing these behaviors.

Which assessment plan is most appropriate?

A) Diagnose social anxiety disorder solely because the student now fears negative evaluation

B) Exclude any neurodevelopmental condition because the student was admitted to college

C) Obtain a detailed developmental history and assess both autism-spectrum features and anxiety before determining the formulation

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Question 10

A client reports hearing accusatory voices and feeling watched during periods of methamphetamine use. The symptoms began after heavy use and have repeatedly resolved after sustained abstinence. The client has no known psychotic symptoms before substance use.

Which diagnostic assessment is most appropriate?

A) Diagnose a primary schizophrenia-spectrum disorder without examining the timing of substance exposure

B) Clarify the course of symptoms during intoxication, withdrawal, and abstinence before distinguishing a substance-induced condition from a primary psychotic disorder

C) Rule out all psychotic disorders because the client recognizes that methamphetamine can affect perception

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Question 11

Cronbach's alpha is most commonly used to estimate which characteristic of a multi-item assessment scale?

A) Predictive validity

B) Internal consistency

C) Interrater reliability

Question 12

What does a larger standard error of measurement indicate about an individual test score?

A) The score has stronger content validity

B) The score represents a higher percentile rank

C) The score is estimated with less precision

Question 13

> Ten minutes into an initial interview, a client asks the counselor to provide a definitive diagnosis and a detailed treatment plan. Only the presenting concern and basic contact information have been discussed.

What is the counselor's best response?

A) Explain the purpose and sequence of the intake, clarify the client's expectations, and gather sufficient biopsychosocial information before offering a formulation

B) Select a diagnosis from the presenting concern so the client does not feel disappointed

C) Begin a detailed intervention plan without assessing history, functioning, strengths, or risks

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Question 14

> Within minutes of an initial appointment, a client begins describing a traumatic event in graphic detail and becomes visibly overwhelmed. The agency's intake normally starts with current needs, safety, service expectations, and a broad history before detailed trauma exploration.

What is the counselor's best response?

A) Require the client to finish a complete trauma narrative so the intake record is comprehensive

B) Stop the client from mentioning trauma at any point during the intake

C) Acknowledge the distress, offer to slow the pace, and explain how the structured intake can gather necessary information without requiring a detailed narrative now

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Question 15

> A 16-year-old is referred after a school employee noticed cannabis vaping. A parent remains in the room and answers most substance-use questions. The adolescent gives brief answers and watches the parent before responding.

What is the most useful next assessment step?

A) Rely on the parent's report because an adolescent's self-report about substance use is inherently invalid

B) Order a comprehensive adult personality inventory before asking additional substance-use questions

C) Explain the limits of privacy, conduct an appropriate private portion of the interview, and combine an adolescent substance-use screen with follow-up questions

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Question 16

> Three weeks after giving birth, a client reports recurrent images of accidentally dropping the baby. The images are unwanted and horrifying, and the client avoids carrying the baby near stairs. The client denies wanting to cause harm and recognizes that the images come from the client's own mind.

Which assessment approach is best?

A) Assess the images' frequency, meaning, associated rituals or avoidance, intent, insight, and any psychotic symptoms before reaching a diagnostic conclusion

B) Conclude that the client is experiencing psychosis because all thoughts involving infant harm are delusions

C) Reassure the client that unwanted thoughts never require further risk or diagnostic assessment

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Question 17

> A client with unpredictable seasonal income assumes that counseling is free. The clinic charges fees, has a late-cancellation policy, and offers a separate application for reduced fees. None of these terms has yet been reviewed with the client.

What should the counselor do before services begin?

A) Delay all discussion of cost until the client receives the first bill

B) Waive every fee without following the clinic's stated assistance process

C) Explain fees, cancellation terms, and available assistance clearly enough for the client to make an informed choice

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Question 18

> A parent schedules an intake for a 20-year-old child, completes the history form, and asks to receive updates after each session. The 20-year-old arrives independently and has not authorized disclosure to the parent.

What should the counselor do during intake?

A) Treat the parent as the client because the parent initiated contact and completed the form

B) Explain confidentiality to the adult client, verify the client's own account and preferences, and obtain authorization before sharing information with the parent

C) Send routine updates to the parent but omit only the details the counselor considers especially sensitive

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Question 19

A bereaved client occasionally dreams of a deceased spouse and, on waking, briefly senses the spouse's comforting presence. The client says this experience is consistent with family and spiritual beliefs, does not command behavior, and causes no impairment or fear.

How should the counselor assess this report?

A) Record a psychotic disorder because sensing a deceased person is always a hallucination requiring diagnosis

B) Ignore the experience entirely because culturally meaningful experiences can never be clinically relevant

C) Explore the experience's cultural meaning, context, duration, distress, impairment, and related symptoms before deciding whether it is pathological

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Question 20

A researcher compares results from a brief depression screener with diagnoses from an established structured diagnostic interview administered at the same time. This comparison primarily evaluates which property of the screener?

A) Concurrent criterion validity

B) Internal consistency

C) Test-retest reliability

Question 21

A counselor is assessing trauma symptoms in a 5-year-old whose caregiver prefers Spanish. The child has limited reading ability and expresses experiences mainly through play and short phrases.

Which assessment strategy is best?

A) Administer a self-report measure designed for English-speaking adults because adult instruments contain more detailed questions

B) Use only unstructured observation because no standardized information can be useful with a young child

C) Combine developmentally appropriate interview and observation with a validated caregiver measure available in the caregiver's preferred language

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Question 22

For more than a year, a client has experienced intrusive fears of contaminating family members and washes for about three hours each day to reduce anxiety. The client recognizes that the fears are excessive but feels unable to resist the rituals. The symptoms substantially disrupt work and family life.

Which diagnosis is best supported by the information?

A) Illness anxiety disorder

B) Obsessive-compulsive disorder

C) Delusional disorder, somatic type

Question 23

> A prospective client asks a counseling clinic for a psychological hardship evaluation to support an immigration petition. The clinic provides treatment but does not conduct forensic or immigration evaluations, and its reports are not designed for legal proceedings.

What is the best intake response?

A) Explain the clinic's treatment role and limits, assess whether the person also seeks counseling, and offer referral information for a qualified independent evaluator

B) Begin therapy and promise that the treating counselor will later write whatever legal opinion the client needs

C) Decline all contact without explaining the service mismatch or offering relevant referral options

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Question 24

> A client reports that a former partner has been following the client to work, recently threatened to break into the home, and previously strangled the client. The former partner owns a firearm and has ignored a request to stop contact. The client is afraid to return home tonight.

What is the counselor's best assessment response?

A) Conduct an immediate danger assessment and collaborate on urgent safety and support options appropriate to the level of risk

B) Focus only on whether the client meets criteria for an anxiety disorder before discussing the former partner's behavior

C) Recommend a routine communication exercise with the former partner to clarify expectations

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Question 25

A client scores above the recommended cutoff on a well-validated screening tool. What is the most accurate interpretation?

A) The client definitively meets diagnostic criteria for the disorder

B) The result indicates that further clinical assessment is warranted

C) The result proves that treatment must begin before an interview occurs

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Question 26

> An adult who uses an augmentative and alternative communication device arrives with a support worker. The counselor directs each question to the worker and moves to the next topic before the client has time to compose responses.

How should the counselor modify the intake?

A) Continue using the worker as the primary respondent because device-assisted answers take longer

B) Ask the worker to decide which topics the client is capable of discussing

C) Address the client directly, ask about communication preferences, allow sufficient response time, and involve the worker only as authorized

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Question 27

A client reports injecting illicit fentanyl daily, surviving an overdose two days ago, and planning to use alone again tonight. The client currently shows no signs of acute intoxication or withdrawal and is willing to consider help.

Which disposition best matches the assessment findings?

A) Facilitate urgent same-day substance-use and medical evaluation while addressing immediate overdose risk and available emergency supports

B) Begin routine weekly counseling and postpone level-of-care decisions until a personality inventory is completed

C) Schedule a reassessment in one month because the absence of current withdrawal indicates low risk

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Question 28

What is a principal advantage of a well-designed structured diagnostic interview?

A) It promotes consistent coverage of relevant criteria while still requiring clinical judgment

B) It guarantees a correct diagnosis regardless of the interviewer's training

C) It eliminates the need to consider culture, context, and functional impairment

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Question 29

Six weeks after giving birth, a client has an elevated score on a depression screener. During interview, the client is tearful and reports guilt, reduced interest, poor concentration, and difficulty functioning, but notes that some sleep disruption occurs when the newborn wakes. The client denies thoughts of self-harm.

How should the counselor use the screening result?

A) Diagnose a depressive disorder from the total score alone without further inquiry

B) Review individual symptoms, timing, functioning, supports, and risk, and integrate the findings with the interview before forming a conclusion

C) Disregard the score because sleep disruption after childbirth makes depression screening uninterpretable

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Question 30

An agency sometimes records sessions for clinical supervision, but recording is optional and requires separate consent. A new client believes recording is mandatory and does not know who could view a recording or when it would be deleted.

What should the counselor do before requesting recording consent?

A) Begin recording because general consent for counseling includes all supervision methods

B) Explain that recording is optional, who may access it, how it will be protected and destroyed, and the effect of declining

C) Avoid discussing supervision because details about recording could influence the client's decision

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PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: C

Correct Answer: Speak directly with the client about the friend's desired role, privacy boundaries, and any specific permission before involving the friend

Explanation: The client's invitation to attend does not automatically authorize ongoing disclosure or decision-making by the friend. The counselor should center the client, define the support role, and obtain specific permission for any future involvement.

Question 2 — Correct: B

Correct Answer: Bipolar II disorder

Explanation: The history includes a major depressive episode and a distinct hypomanic episode without any manic episode. The absence of marked impairment, psychosis, or hospitalization during the elevated period supports hypomania rather than mania under the facts provided.

Question 3 — Correct: C

Correct Answer: Provide accessible explanations, assess the client's understanding and decision-making ability, and verify whether anyone has legal authority before involving a substitute decision-maker

Explanation: A diagnosis or support need does not by itself establish incapacity. The counselor should support the client's comprehension and assess actual decision-making ability while verifying, rather than assuming, the sister's authority.

Question 4 — Correct: B

Correct Answer: Circumstantial

Explanation: Circumstantial thought includes excessive but logically connected detail before the person eventually reaches the point. Flight of ideas involves rapid shifts among topics, whereas thought blocking is an abrupt interruption in the train of thought.

Question 5 — Correct: B

Correct Answer: Explore scheduling and child-care barriers with the client and determine whether the agency's available supports make services workable

Explanation: Intake should identify concrete access barriers and collaboratively evaluate whether available supports make participation feasible. Prior missed appointments under different conditions do not establish unwillingness, and imposing another unworkable time would not assess suitability.

Question 6 — Correct: A

Correct Answer: Explain the clinic's scope, check for urgent concerns and interest in counseling, and offer an appropriate prescribing referral

Explanation: The clinic should accurately describe its nonprescribing role while determining whether urgent needs or counseling interests are present. A referral can address medication services without misrepresenting the counselor's scope or abruptly dismissing the person.

Question 7 — Correct: A

Correct Answer: Performance in relation to defined knowledge or skill standards

Explanation: Criterion-referenced scores indicate performance against specified objectives or standards. Norm-referenced scores instead describe standing relative to a comparison group.

Question 8 — Correct: A

Correct Answer: Use multiple methods and qualified language support while considering educational opportunity, language development, culture, and measures appropriate for the student's background

Explanation: Assessment should distinguish a possible learning difficulty from language acquisition and unequal educational opportunity using appropriate, multiple sources. An inadequately normed English-only test could produce misleading conclusions, while indefinite delay would also withhold needed evaluation.

Question 9 — Correct: C

Correct Answer: Obtain a detailed developmental history and assess both autism-spectrum features and anxiety before determining the formulation

Explanation: Lifelong social-communication differences, restricted interests, sensory sensitivity, and rigidity warrant neurodevelopmental assessment. Current fear of criticism may also support anxiety, so the counselor should evaluate both rather than force an either-or conclusion prematurely.

Question 10 — Correct: B

Correct Answer: Clarify the course of symptoms during intoxication, withdrawal, and abstinence before distinguishing a substance-induced condition from a primary psychotic disorder

Explanation: The timing of psychotic symptoms in relation to methamphetamine exposure and resolution during abstinence is central to the differential. Substance involvement does not justify an automatic primary diagnosis or exclusion of all psychotic disorders.

Question 11 — Correct: B

Correct Answer: Internal consistency

Explanation: Cronbach's alpha estimates how consistently items on a multi-item scale relate to the same construct. It does not measure agreement between raters or determine how well a score predicts an external outcome.

Question 12 — Correct: C

Correct Answer: The score is estimated with less precision

Explanation: The standard error of measurement reflects uncertainty around an observed score. A larger value produces a wider likely range for the person's underlying score and therefore indicates less precision.

Question 13 — Correct: A

Correct Answer: Explain the purpose and sequence of the intake, clarify the client's expectations, and gather sufficient biopsychosocial information before offering a formulation

Explanation: A defensible formulation and plan require enough information about history, functioning, strengths, risks, and goals. Clarifying the process respects the client's request without making a premature diagnosis or beginning an unsupported plan.

Question 14 — Correct: C

Correct Answer: Acknowledge the distress, offer to slow the pace, and explain how the structured intake can gather necessary information without requiring a detailed narrative now

Explanation: The counselor can validate the client's experience and maintain the purpose of intake without encouraging overwhelming exposure. A detailed trauma narrative is not required to establish current needs and safety, but banning all mention of trauma would also be unresponsive.

Question 15 — Correct: C

Correct Answer: Explain the limits of privacy, conduct an appropriate private portion of the interview, and combine an adolescent substance-use screen with follow-up questions

Explanation: A private, developmentally appropriate portion of the assessment can improve disclosure while preserving stated confidentiality limits. A validated adolescent screen should inform, not replace, follow-up questions and information from other relevant sources.

Question 16 — Correct: A

Correct Answer: Assess the images' frequency, meaning, associated rituals or avoidance, intent, insight, and any psychotic symptoms before reaching a diagnostic conclusion

Explanation: Unwanted, ego-dystonic infant-harm images with preserved insight may occur in an obsessive presentation, but risk and psychotic symptoms still require direct assessment. Neither automatically labeling the experience psychosis nor dismissing it without further inquiry is justified.

Question 17 — Correct: C

Correct Answer: Explain fees, cancellation terms, and available assistance clearly enough for the client to make an informed choice

Explanation: Financial obligations and material attendance policies should be understood before the client agrees to services. The counselor should also describe the established assistance option rather than conceal costs or promise an unauthorized waiver.

Question 18 — Correct: B

Correct Answer: Explain confidentiality to the adult client, verify the client's own account and preferences, and obtain authorization before sharing information with the parent

Explanation: The 20-year-old is the prospective adult client and controls permission for routine disclosure. A parent's scheduling or completion of a form does not automatically grant access to counseling information.

Question 19 — Correct: C

Correct Answer: Explore the experience's cultural meaning, context, duration, distress, impairment, and related symptoms before deciding whether it is pathological

Explanation: Bereavement experiences must be interpreted within cultural and spiritual context as well as their clinical features. The absence of impairment, fear, commands, or broader symptoms argues against reflexively assigning a psychotic diagnosis, but the experience should still be understood rather than ignored.

Question 20 — Correct: A

Correct Answer: Concurrent criterion validity

Explanation: Concurrent criterion validity concerns how well scores agree with an appropriate criterion measured at about the same time. Internal consistency concerns relationships among items, while test-retest reliability concerns stability across administrations.

Question 21 — Correct: C

Correct Answer: Combine developmentally appropriate interview and observation with a validated caregiver measure available in the caregiver's preferred language

Explanation: A young child's assessment should use methods suited to developmental and communication abilities while incorporating informed caregiver data. An adult self-report is inappropriate, and standardized information need not be abandoned when a suitable validated measure exists.

Question 22 — Correct: B

Correct Answer: Obsessive-compulsive disorder

Explanation: The presentation includes intrusive contamination obsessions and time-consuming washing compulsions performed to reduce anxiety, with substantial impairment. Preserved recognition that the fears are excessive is inconsistent with the fixed belief expected in delusional disorder.

Question 23 — Correct: A

Correct Answer: Explain the clinic's treatment role and limits, assess whether the person also seeks counseling, and offer referral information for a qualified independent evaluator

Explanation: The requested legal evaluation falls outside the clinic's stated role, so the mismatch should be explained before services begin. The clinic can still assess interest in treatment and provide an appropriate independent referral without promising a forensic opinion.

Question 24 — Correct: A

Correct Answer: Conduct an immediate danger assessment and collaborate on urgent safety and support options appropriate to the level of risk

Explanation: Escalating stalking, prior strangulation, threats, firearm access, and fear of returning home indicate substantial potential danger. Diagnostic labeling or routine communication work should not displace immediate assessment and safety planning.

Question 25 — Correct: B

Correct Answer: The result indicates that further clinical assessment is warranted

Explanation: A screening cutoff identifies elevated likelihood or concern rather than establishing a diagnosis. Diagnostic conclusions and treatment decisions require integration with interview data, impairment, context, and relevant exclusions.

Question 26 — Correct: C

Correct Answer: Address the client directly, ask about communication preferences, allow sufficient response time, and involve the worker only as authorized

Explanation: Use of an augmentative device does not shift client status or decision-making to a support worker. Direct communication, adequate time, and the client's stated preferences produce a more valid and respectful intake.

Question 27 — Correct: A

Correct Answer: Facilitate urgent same-day substance-use and medical evaluation while addressing immediate overdose risk and available emergency supports

Explanation: Daily fentanyl injection, a very recent overdose, and planned solitary use indicate high near-term overdose risk even without current withdrawal. Urgent evaluation and immediate risk-reduction supports are more appropriate than routine or delayed reassessment.

Question 28 — Correct: A

Correct Answer: It promotes consistent coverage of relevant criteria while still requiring clinical judgment

Explanation: Structure can improve consistency and reduce omission of important diagnostic criteria. It does not guarantee accuracy or eliminate the need to interpret culture, context, impairment, and other clinical information.

Question 29 — Correct: B

Correct Answer: Review individual symptoms, timing, functioning, supports, and risk, and integrate the findings with the interview before forming a conclusion

Explanation: A screening score identifies the need for fuller assessment and should be interpreted alongside symptom detail, context, functioning, and risk. Normal postpartum sleep interruption can complicate one symptom but does not make the entire result diagnostic or meaningless.

Question 30 — Correct: B

Correct Answer: Explain that recording is optional, who may access it, how it will be protected and destroyed, and the effect of declining

Explanation: Separate recording consent must be informed and voluntary under the agency's stated policy. The client needs accurate information about purpose, access, security, retention, and the consequence of declining before deciding.
